

Table 2: Imaging features in different patterns of Krabbe disease.

Pattern	Number of MRIs	Confluent deep and periventricular T2W hyperintensity with PLIC and brainstem CST involvement	Non-confluent deep and periventricular T2W hyperintensity with PLIC involvement	Isolated corticospinal tract T2W hyperintensity	Isolated posterior periventricular T2W hyperintensity with (+)/without (-) corticospinal tract involvement	Corpus callosal involvement	Thalamic volume loss and T2W hypo/hyperintensity	Confluent cerebellar white matter T2W hyperintensity	Dentate hilum T2W hyperintensity	Basal ganglia atrophy/hyperintensity	Optic nerve thickening	Cerebral atrophy	Cerebellar atrophy	Brainstem atrophy midbrain (M), pons (P)
I	18	18	0	0	0	12 ^a 2 ^c 12 ^d	13 ^c 15 ^b 4 ^e	18	18	Caudate atrophy-8 Caudate hyperintensity -1 Putamen hypointensity -3	16	7 s 7 mo 2 mil 2 n	4 mil	M s-8 M mil-9 M n-1 P s-9 P mil-8 P n-1
II	12	0	0	0	11 + 1 ^c	9 ^a	0	0	0	0	0	Parietofrontal atrophy -3	0	0
III	5	0	0	5	0	5 ^c	1 ^{b,g}	0	0	0	0	5 perirolandic	0	0
IV	3	0	2	0	0	2 ^b 1 ^a	3 ^{ca}	0	3	0	0	1 mil	1 mil	0

Corpus Callosal Involvement: ^aDiffuse splenial hyper intensity, ^bFocal splenial hyper intensity, ^cGenu hyper intensity, ^dDiffuse atrophy, ^eFocal posterior body atrophy, Thalamus: ^fHypo intensity, ^gHyperintensity.

^hAtrophy, cerebral/cerebellar/brainstem atrophy s-severe, mo-Moderate, mil- Mild, n-No atrophy. PLIC: Posterior limb of internal capsule, CST: Corticospinal tract